SOAP Note							
Pt Name:				Age:		Emergency contact #:	
Airway:		Breathing:		Circulation:		(neuro)Deficits:	Environment:
If Trauma, start with head to toe. Note findings below:						**If Medical, start with SAMPLE**	
Head/Face/Neck						Signs/Symptoms (OPQRST):	
Chest/Shoulders						Allergies:	
Abdomen/Pelvis						Medications:	
Arms/Legs						Past History:	
Back/Spine						Last ins/outs:	
Vitals:						Events prior:	
Time	LOR	HR	RR	Pupils	Skin	**Verbal report:** "I have a __ year old patient with a chief complaint of ___. First set of vitals were ___. Most recent set of vitals were ___. (mention relevant SAMPLE information and any treatments given)	

SOAP Note							
Pt Name:				Age:		Emergency contact #:	
Airway:		Breathing:		Circulation:		(neuro)Deficits:	Environment:
If Trauma, start with head to toe. Note findings below:						**If Medical, start with SAMPLE**	
Head/Face/Neck						Signs/Symptoms (OPQRST):	
Chest/Shoulders						Allergies:	
Abdomen/Pelvis						Medications:	
Arms/Legs						Past History:	
Back/Spine						Last ins/outs:	
Vitals:						Events prior:	
Time	LOR	HR	RR	Pupils	Skin	**Verbal report:** "I have a __ year old patient with a chief complaint of ___. First set of vitals were ___. Most recent set of vitals were ___. (mention relevant SAMPLE information and any treatments given)	

ABCDE's **Airway** management: Look in mouth, clear obstructions **Breathing** adequacy: Look, listen, and feel for compromise **Circulation**: Assess for pulse & major bleeding; control bleeding, treat for shock. **Deficit**: Maintain manual stabilization of the spine unless patient has no significant MOI. Assess for deficiency. **Environment**: Assess and treat environmental hazards;	**SAMPLE** **Symptoms**: ex: Headache? Dizziness? Nausea? **Allergies**: to medications, OTC's, Foods, Insects, Pollens **Medications**: Prescription, OTC's, Alcohol or recreational drugs **Pertinent Medical History**: Medical history that relates **Last Intake/Output**: Food/Water; Urination, Vomiting **Events**: Events leading up to incident/illness
AVPU Scale (use for LOC's—Level of Consciousness) **AOx4:** Alert & Oriented to Person, Place, Time & Events **AOx3:** Alert & Oriented to Person, Place & Time **AOx2:** Alert & Oriented to Person & Place **AOx1:** Alert & Oriented to Person **V**: Verbally responsive – responds to verbal stimuli **P**: Painfully Responsive – responds to painful stimuli **U**: Unresponsive – does not respond to any stimuli	**OPQRST** **Onset:** Was the onset sudden or gradual? **Provokes/Palliates:** What makes it worse? Better? **Quality:** Describe the pain, sharp vs dull; constant vs erratic **Radiates/Refers:** Does the sensation move anywhere? **Severity:** How does this rate on a scale of 1 -10? **Time:** How long has it been going on?
Head to Toe – When performing a head to toe exam you want to careful examine & palpate each body section for hidden injuries. Don't be too gentle! You might not find an injury if you are too gentle. Make sure to remove/move clothing as necessary. You want to get down to skin in injured or possibly injured areas.	**Vital Signs** **LOC's:** See AVPU scale. **Heart Rate (HR):**regular/irregular, strong/weak.60-100 avg **Respiratory Rate (RR):** labored/unlabored. 12-20 average **Pupils:** PERRL (Pupils Equal, Round & Reactive to Light) **Skin (SCTM):** Skin color, temperature, moisture. Pink, Warm, Dry is normal

ABCDE's **Airway** management: Look in mouth, clear obstructions **Breathing** adequacy: Look, listen, and feel for compromise **Circulation**: Assess for pulse & major bleeding; control bleeding, treat for shock. **Deficit**: Maintain manual stabilization of the spine unless patient has no significant MOI. Assess for deficiency. **Environment**: Assess and treat environmental hazards;	**SAMPLE** **Symptoms**: ex: Headache? Dizziness? Nausea? **Allergies**: to medications, OTC's, Foods, Insects, Pollens **Medications**: Prescription, OTC's, Alcohol or recreational drugs **Pertinent Medical History**: Medical history that relates **Last Intake/Output**: Food/Water; Urination, Vomiting **Events**: Events leading up to incident/illness
AVPU Scale (use for LOC's—Level of Consciousness) **AOx4:** Alert & Oriented to Person, Place, Time & Events **AOx3:** Alert & Oriented to Person, Place & Time **AOx2:** Alert & Oriented to Person & Place **AOx1:** Alert & Oriented to Person **V**: Verbally responsive – responds to verbal stimuli **P**: Painfully Responsive – responds to painful stimuli **U**: Unresponsive – does not respond to any stimuli	**OPQRST** **Onset:** Was the onset sudden or gradual? **Provokes/Palliates:** What makes it worse? Better? **Quality:** Describe the pain, sharp vs dull; constant vs erratic **Radiates/Refers:** Does the sensation move anywhere? **Severity:** How does this rate on a scale of 1 -10? **Time:** How long has it been going on?
Head to Toe – When performing a head to toe exam you want to carefully examine & palpate each body section for hidden injuries. Don't be too gentle! You might not find an injury if you are too gentle. Make sure to remove/move clothing as necessary. You want to get down to skin in injured or possibly injured areas.	**Vital Signs** **LOC's:** See AVPU scale. **Heart Rate (HR):**regular/irregular, strong/weak.60-100 avg **Respiratory Rate (RR):** labored/unlabored. 12-20 average **Pupils:** PERRL (Pupils Equal, Round & Reactive to Light) **Skin (SCTM):** Skin color, temperature, moisture. Pink, Warm, Dry is normal